Nivolumab · every 2 weeks · adjuvant melanoma

Nivolumab helps immune cells act against melanoma. After complete surgery, its aim is to lower the risk of recurrence; this does not guarantee that melanoma will not return.

Completely resected melanoma with lymph-node involvement or metastatic disease; the team confirms the stage III or resected stage IV indication. Uveal melanoma is excluded from this adjuvant pathway. · NCCP 00483 v13a · Document revised 9 November 2026

Your personal plan: this is the standard regimen schedule. Your team confirms your prescribed doses, cycles, treatment changes and appointments in your written plan.

Completely resected melanoma with lymph-node involvement or metastatic disease; the team confirms the stage III or resected stage IV indication. Uveal melanoma is excluded from this adjuvant pathway. This information supports individual consent; it does not issue or change a prescription.

My overall treatment

Every 2 weeks; up to 12 months after surgery.

14 day view

Treatment timing in this window

Nivolumab treatment — Day 1

Adjuvant treatment after surgery

Nivolumab is given as 240 mg by intravenous (IV) infusion over 30 minutes OR 600 mg by subcutaneous injection over 3–5 minutes, on Day 1 every 2 weeks. Allow extra time for assessment, preparation and observation. The team selects one formulation. A subcutaneous injection is given into the thigh or abdomen, alternating sites and avoiding damaged skin, scars or moles. IV and subcutaneous doses are not interchangeable milligram for milligram. Adjuvant treatment continues for up to 12 months, subject to safety review.

A typical 14-day cycle

Day 1	Day 2–14
Treatment visit	Between visits

Timing may change after review. Side effects can occur on any day; there are no guaranteed safe days.

My tests and checks

When	What is checked	Timing / reason
Before	**Baseline safety** NCCP	Blood count, kidneys/liver, glucose and thyroid. BRAF tumour status.
During	**Before each dose** NCCP	Blood count, kidneys/liver and glucose. Thyroid once a month. Cortisol if indicated.
After / response	**Follow-up** NCCP / individual plan	Immune monitoring for at least 5 months after stopping. Scans / examinations individually planned.

Between treatments

- Immune symptoms can occur between visits or after treatment stops. Report changes promptly.
- Carry your immunotherapy alert card; follow prescribed steroids or hormone replacement.
- If your team has permanently stopped nivolumab because of a serious immune reaction, improvement does not automatically make restarting safe. Keep that decision in your treatment record; only your specialist team can review further treatment.

Bring the written medicine plan, test dates and actual oncology/out-of-hours contacts.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Hormones / head / eyes

Nivolumab

New severe headache, visual change, unusual tiredness, dizziness, thirst or frequent urination may signal immune effects and needs prompt advice.

3 Heart / muscles

Nivolumab

Chest pain, palpitations, marked muscle weakness or swallowing difficulty can be serious immune effects. Seek urgent assessment.

5 Kidneys / urine

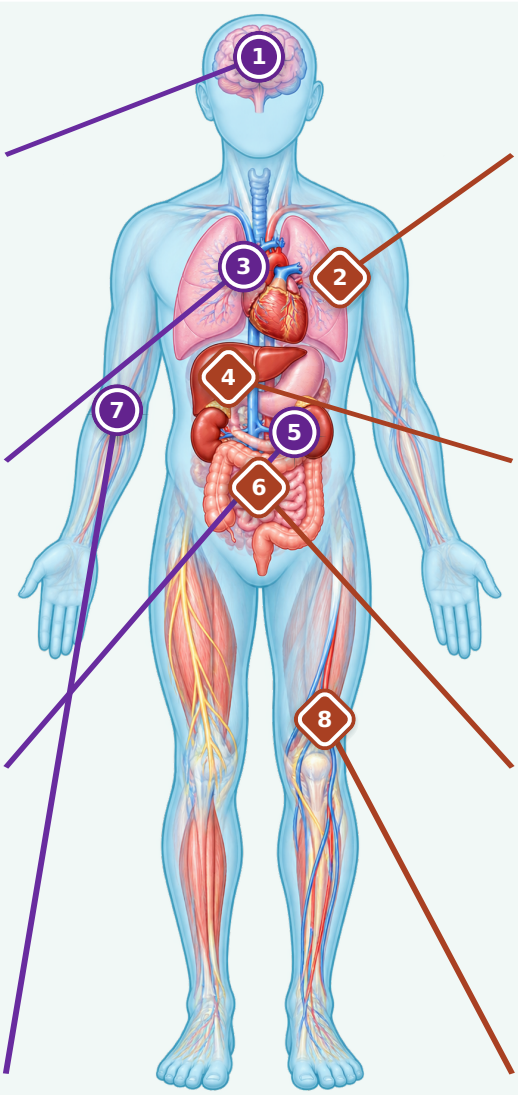
Nivolumab

Kidney inflammation may be detected on blood tests. Report reduced urine, swelling, blood in urine or a marked change from normal.

7 Skin / mouth

Nivolumab

Report rash or itching early. Blistering, peeling, mouth or genital sores, or rash with fever needs urgent assessment.



2 Lungs / breathing

Nivolumab

New cough or breathlessness may reflect lung inflammation or another serious cause. Call promptly; severe breathing difficulty is an emergency.

4 Liver

Nivolumab

Inflammation may first show in blood tests. Report yellow eyes or skin, dark urine, unusual itching or upper-abdominal pain.

6 Bowels / abdomen

Nivolumab

Report increased stool frequency, diarrhoea, blood or mucus, or abdominal pain early. Severe pain, black stools or dehydration needs urgent help.

8 Nerves / strength

Nivolumab

New tingling, weakness, unsteadiness, facial change or trouble swallowing needs prompt review. Sudden weakness or speech difficulty is an emergency.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Immunotherapy can cause inflammation in healthy organs during treatment or after it stops. Contact the oncology team promptly for new diarrhoea, blood in stools, cough or breathlessness, yellow skin or eyes, severe headache, vision changes, marked thirst, unusual weakness or a spreading rash. Tell any clinician that you are receiving or have received immunotherapy.

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Keep your treatment details to hand

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Print this page **one-sided on A4 at Actual size / 100%**. Cut only the dashed outer border around both panels. **Do not cut the centre line.** Fold with FRONT and BACK facing out, then glue or tape the blank backs together. Finished size: **85.6 × 54 mm**. Keep the stamp side writable.

FRONT / CONTACTS

My Treatment Card

SACTCheck

Name: _____

Nivolumab · melanoma

Every 2 weeks; up to 12 months after surgery.

Phase: _____ Cycle plan: team to confirm

Daytime (EXAMPLE) 087 12345678

Out of hours (EXAMPLE) 087 87654321

Unwell or fever? Call oncology now.
Severe breathing difficulty, chest pain or collapse: **112 / 999.**

EXAMPLE NUMBERS — replace before patient use.

BACK / TREATMENT RECORD

My cycle record

Rolling record; planned adjuvant finish: _____

Cycle	Date / initials

My treatment information

Latest treatment date: _____

Plan changed? Ask the team to update this card.

CUT outer border

FOLD centre — do not cut

Keep the record accurate

The team dates and initials each recorded treatment unit when complete. A cycle is complete after its final planned dose; a weekly-treatment stamp records that weekly dose. Delays, omitted doses or a changed plan need a note or a replacement card. A stamp does not approve the next treatment. Keep any medicine-specific alert card too.

Put your treatment page on your phone

Scan your QR and check the treatment name. Save that page to your home screen.

iPhone / Safari

Tap Share → Add to Home Screen. Select Open as Web App if offered, then Add.

Android / Chrome

Tap the three-dot menu → Add to Home screen or Install and create shortcut. Choose the shortcut option and confirm.

Menu wording varies. The QR opens information, not a personal record. Paper stamps do not update the website. Check that the saved icon opens the correct treatment.

Your local contacts and resources

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

Hospital / team: _____

Backup if the team cannot be reached: _____

Additional local services and resources will be added when confirmed. Your team must check and complete this page before patient use.

Feasibility build. Clinical, pharmacy and patient/PPI review pending. Your team confirms your personal plan.